



Developing ED Management Plans for Patients with Complex Care Needs and/or Frequent Presentations to Hospital Procedure

1. Scope

Site	Operational Area	Applicable to
Armada Kalamunda Group	Emergency Departments	All clinical staff

2. Purpose

This procedure describes the process for identifying, assessing, developing and implementing a management plan for patients who have complex care needs and/or present frequently to hospital. The aim of the ED Management Plan is to provide current and easily accessible patient information to facilitate health care professionals to make informed decisions for the ongoing care needs of the patient.

3. Procedure

Procedure for implementing a management plan – see [Appendix 1](#) for a flowchart of the process:

1. Patients in need of a management plan in the Emergency Department (ED) are identified by medical, nursing or allied health staff. This includes:
 - Patients with ongoing complex care needs where an ED plan could prevent further hospital admissions.
 - Patients that frequently present to hospital when all medical investigations have been concluded and/or all community supports are in place and/or patient recurrently declines further input.
 - ED patients requiring a behavioural management plan.
2. An ED Management Plan is written by the relevant health professionals and should include the contact details relating to the patient and all relevant specialists or community services.
3. The development of the Management Plan should involve all relevant key stakeholders as appropriate; this may include the consultant, nursing staff, multidisciplinary team, other hospitals, the patient and/or the carers and the patient's General Practitioner (GP)
4. The Management Plan is reviewed by the relevant team members and updated as required. It should include copies of related documentation available such as advance health directives, enduring power of attorney or guardianship.
5. The final agreed Management Plan is to be signed off by relevant senior key stakeholder staff, for example the patient's Medical Consultant, Director of General Medicine, Director of Emergency Medicine, Duty Officer Consultant in the ED and/or the Emergency Department Nurse Unit Manager (ED NUM) as soon as possible.
6. The signed management plan will be sent to ED NUM to upload as an external register alert on the Emergency Department Information System (EDIS). The original master

management plan will be filed in the patient's medical record and a copy in the ED management plan file.

3.1 Review of Management Plans:

It is the responsibility of the department or service that created the management plan to review each plan to ensure the details remain current and applicable.

3.1.1 Emergency Department

- Management plans are to be reviewed as clinically indicated or once a year by the ED Associate NUM.
- Management plans are to remain in place for a maximum of five years.

4. Legislative context

The following documents are required to give effect to this procedure [i.e. the documents included are mandatory]:

- Health Department of Western Australia (HDWA) [Clinical Handover Policy MP0095/18](#)
- East Metropolitan Health Service (EMHS) [Patient Identification and Procedure Matching Policy \(EMHS: 21\)](#)

5. Supporting information

The following documents support the implementation of this procedure:

- [Discharge of Vulnerable Emergency Department Patients Procedure](#)
- [Clerical Discharge Procedures](#)
- [Emergency Department Allied Health Team Assessment, Intervention and Discharge Planning](#)
- [AKMR 6.5 Patient Management Plan](#)

6. Compliance, monitoring and evaluation

Compliance for this procedure will be evaluated through audits to ensure a minimum 80% compliance in the following indicators:

6.1 Emergency Department

- Signed by the appropriate senior key stakeholder.
- Management plan alert on EDIS.
- Up to date copy of the patient's management plan in the ED file.
- Reviewed as clinically indicated or once a year.
- Management plans in place for a maximum of five years.

7. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 2 – Partnering with Consumers
- Standard 6.3 – Clinicians use organisational processes from the Partnering with Consumers
- Standard 6.8 – Clinicians use structured clinical handover processes that include:

8. References

1. Western Health Victoria, 2011, Hospital Admission Risk Program: Monitoring and Evaluation Framework. State Government Victoria.
2. Integrated Care Monitoring and Evaluation, 2017, Risk Stratification, User Guide for the Adapted Hospital Admission Risk Prediction Tool. NSW Ministry of Health.

9. Glossary

The following definitions are relevant to this policy.

Term	Definition
Frequent presentation	Patient has four or more unplanned ED presentations to a WA health hospital in the past 12 months.
Complex care need	Patients with substantial and ongoing healthcare needs that require a coordinated approach across multiple disciplines. The care of these patients is often outside the standard model of care.

10. Policy owner (relating to these procedures)

Director Allied Health and Ambulatory Care

Enquiries relating to this procedure may be directed to:

Title: Team Leader
Department: HealthCare to Community
Email: AHS.CareCoordination@health.wa.gov.au

11. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V2	18/07/2022	18/07/2025	Minor Amendments

12. Authorisation

This procedure has been authorised and issued by the Executive Director.

Authorised by	A/Director Allied Health
Authorisation date	06/05/2022
Published date	18/07/2022

Appendix 1: Flow chart for developing management plans for medical patients

